

YELLOW REPORT

In de novo HR+/HER2– lobular breast cancer with ovarian metastases, **primary tumor resection does not improve overall survival** compared with systemic therapy alone [1]. The only consistent benefit is improved **local control** [2], which may be relevant in select patients with symptomatic or high-risk primaries [3]. Given the asymptomatic, low-burden disease in this case, **systemic therapy alone is preferred** [3]; reserve surgery for palliative indications or clinical trials, and manage her high motivation through counseling on limited benefit and potential harms [2].

Evidence on primary tumor resection in de novo HR+/HER2– lobular breast cancer

- **Randomized controlled trials (RCTs):** Multiple RCTs and meta-analyses show no OS benefit with primary tumor resection in de novo metastatic breast cancer, including HR+/HER2– subtypes [1].
- **Local control benefit:** Surgery consistently improves local control, but this does not translate into OS improvement [2] [4].
- **Selection bias:** Retrospective studies suggesting benefit are confounded by selection bias; fitter patients with less disease burden are more often offered surgery [4] [5].

Specific considerations for lobular histology

Lobular breast cancer has a **unique biology** with a higher propensity for bone and gastrointestinal metastases and a more indolent course; however, this does not confer a unique survival benefit from primary resection [6]. In this case, the **discordant PR status** (PR– in the primary vs PR+ in ovarian metastases) raises the possibility of primary-tumor PR false-negativity or tumor heterogeneity; in the absence of discordant ER/HER2, current guideline focus remains on ER/HER2 status to direct therapy [3] [7] [8].

Clinical guidelines and expert consensus

- **NCCN:** Systemic therapy is first-line for de novo metastatic disease; surgery is reserved for palliation or clinical trials [3].
- **ASCO:** Recommends multidisciplinary discussion and selective use of surgery in limited disease or bone-only metastases, but emphasizes that OS benefit is unproven [9] [10].
- **ESMO:** Similar to NCCN, emphasizing no proven OS benefit and selective use in carefully chosen patients [9].

Patient-specific factors influencing surgical decision-making

Factor	Influence on surgical decision
Age and performance status	Younger, fit patients are more likely to be considered for surgery, but age alone does not confer benefit [9] [10]
Disease burden	Limited metastatic burden (e.g. bone-only) may be considered for surgery, but OS benefit remains unproven [9] [10]
Patient preference	High motivation for surgery should be addressed with counseling on limited benefit and potential harms [2]
Symptomatic primary tumor	Surgery is indicated for palliation (e.g. ulceration, bleeding) [3]

Risks and benefits of primary tumor resection

- **Benefits:** Improved local control, potential quality of life improvement in select patients, and psychological benefit for some patients [2].
- **Risks:** Surgical complications, delay in systemic therapy, and potential impact on quality of life without OS benefit [2].
- **Evidence level:** High-quality evidence shows no OS benefit, moderate-quality evidence shows improved local control [2].

Alternative strategies and supportive care

Given the lack of OS benefit, **prioritize high-quality systemic therapy** (endocrine therapy ± CDK4/6 inhibition, with PIK3CA-directed therapy in selected settings) and supportive care [3] [7]. If the primary becomes symptomatic or the patient remains distressed, consider palliative surgery or enrollment in clinical trials [3].

Summary of recommendations

- **Standard care:** Systemic therapy alone is preferred for de novo HR+/HER2– lobular breast cancer with ovarian metastases [3].
- **Surgery:** Reserve for palliative indications, clinical trials, or carefully selected patients after multidisciplinary discussion and counseling [9] [10].
- **Counseling:** Address patient motivation by explaining the limited benefit and potential harms of surgery [2].

Sources:

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6. [Metastatic HER2-Negative](#). NCI, 2026
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8. [Systemic treatment of patients with metastatic breast cancer: ASCO resource-stratified guideline](#). JCO Global Oncology, 2024
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